



The ASAM Criteria® Navigator

InterQual® 2023 January Release Clinical Revisions

Release date: January 5, 2024

Revisions sections

The following sections describe the criteria revisions for this release.

[Revisions to criteria](#)

[Revisions to medical codes](#)

Revisions to criteria

All medically managed levels of care (1.7, 2.7, 3.7, 4)		
Component	Revision	Rationale
DIM 1, Intoxication and withdrawal Question	Answer choice “Current or anticipated mild signs/symptoms of intoxication or withdrawal can be addressed in any level of care and require prompt evaluation but NOT integrated medication management” has been changed to “Current or anticipated mild signs/symptoms of intoxication or withdrawal can be addressed in any level of care and may require prompt medical evaluation but NOT integrated medical management”	To clarify that the capability to perform a prompt evaluation should be available if required

Levels 3.7 and 4		
Component	Revision	Rationale
DIM 1, Intoxication and withdrawal Question	Answer choice “Current or anticipated moderately severe to severe signs/symptoms of intoxication or withdrawal and patient does NOT require IV medications, but DOES require after-hours medical management and/or nursing care (e.g., due to co-occurring physical or mental health needs that complicate withdrawal management; moderately severe alcohol or sedative-hypnotic withdrawal with history of complications)” has been changed to “Current or anticipated moderately severe to severe signs/symptoms of intoxication or withdrawal and patient does NOT require IV fluids or medications, but DOES require after-hours medical management and/or nursing care (e.g., due to co-occurring physical or mental health needs that complicate withdrawal management; moderately severe alcohol or sedative-hypnotic withdrawal with history of complications)” Answer choice “Current or anticipated moderately severe to severe signs/symptoms or intoxication or withdrawal require IV medications but NOT an acute care setting” has been changed to “Current or anticipated moderately severe to severe signs/symptoms or intoxication or withdrawal require IV fluids and/or medications but NOT an acute care setting”	To clarify that the need for both IV fluids and/or IV medications requires a minimum of Level 3.7 BIO

Levels 1.7, 2.7, and 3.7

Component	Revision	Rationale
DIM 1, Addiction medication needs Question	Answer choice “Patient will initiate MOUD and no complications are expected; AND/OR patient will initiate other addiction medications and require more than weekly medical management to stabilize SUD symptoms and prevent recurrence (e.g., due to history of poor outcomes with clinically managed care)” has been changed to “Patient will initiate or titrate MOUD and no complications are expected; AND/OR patient will initiate or titrate other addiction medications and require more than weekly medical management to stabilize SUD symptoms and prevent recurrence (e.g., due to history of poor outcomes with clinically managed care)”	To clarify that patients who have started MOUD may still need integrated medication management to titrate these medications.

Levels 3.7 and 4		
Component	Revision	Rationale
DIM 2, Physical Health Question	Answer choice “Moderately severe to severe health problems and patient requires daily medical management and/or nursing care, including after-hours (e.g., due to exacerbation by intoxication/withdrawal; lack of sufficient home monitoring); but does NOT require an acute care setting, medications, or wound vacuum-assisted closure” has been changed to “Moderately severe to severe health problems and patient requires daily medical management and/or nursing care, including after-hours (e.g., due to exacerbation by intoxication/withdrawal; lack of sufficient home monitoring); but does NOT require an acute care setting, IV fluids or medications, or wound vacuum-assisted closure” Answer choice “ Moderately severe to severe physical health problems require IV medications and/or wound vacuum-assisted closure, but do NOT require an acute care setting” has been changed to “Moderately severe to severe physical health problems require IV fluids/medications and/or wound vacuum-assisted closure, but do NOT require an acute care setting”	To clarify that the need for IV fluids and/or IV medications requires a minimum of Level 3.7 BIO.

All levels of care		
Component	Revision	Rationale
DIM 3 , Psychiatric and cognitive conditions Question	Question "DIM 3, Psychiatric and cognitive conditions" has been changed to "DIM 3, Active psychiatric symptoms" Answer choice "No specific treatment needs for active psychiatric signs/symptoms or persistent mental health/cognitive disability" has been changed to "No specific treatment needs for active psychiatric signs/symptoms" References to persistent disability have been removed from all answer choices "...mental health signs/symptoms and/or persistent mental health-related disability require ...skilled mental health interventions... and/or assistance with function to permit..." have been changed to "...mental health signs/symptoms require.... skilled mental health interventions....to permit..."	To assess a patient's minimal level of care needs for the DIM 3 "Active psychiatric symptoms" subdimension

All levels of care		
Component	Revision	Rationale
NEW DIM 3 , Persistent disability Question	Added new question "DIM 3, Persistent disability, Choose one: A. Persistent mental health-related or cognitive disability requires specialized assistance and/or interventions to permit effective participation in addiction treatment B. Persistent mental health-related or cognitive disability does NOT require specialized assistance and/or skilled interventions to permit effective participation in addiction treatment C. No persistent mental health or cognitive disability D. Other clinical information"	To assess a patient's minimal level of care needs for the DIM 3 "Persistent disability" subdimension
Levels 1.5, 2.1, 2.5, 3.1, and 3.5		
Component	Revision	Rationale
DIM 3 , Active psychiatric symptoms Question	Answer choice "Active psychiatric signs/symptoms require urgent or frequent (more than weekly) medical or psychiatric management and/or nursing care; AND/OR patient is unable to reliably access external mental health care" has been changed to "Active psychiatric signs/symptoms require urgent or frequent (more than weekly) medical or psychiatric management and/or psychiatric nursing care"	To clarify that this answer choice refers to specialized psychiatric nursing (i.e., medication management from a psychiatric nurse practitioner)

Levels 2.5, 2.7, 3.1, 3.5, and 3.7		
Component	Revision	Rationale
DIM 4, Structure and skills Question	Answer choice “Patient has sufficient daily structure and relapse prevention skills to avoid risky substance use/SUD-related behaviors while in the community during the day, but cannot reliably do so overnight” has been changed to “Patient has sufficient daily structure and relapse prevention skills to avoid risky substance use/SUD-related behaviors while in the community during the day, but cannot reliably do so overnight (and a recovery residence will not provide sufficient support or is not available)”	To clarify that this answer choice refers to the need for a residential level of care

Levels 2.5, 2.7, 3.1, 3.5, and 3.7		
Component	Revision	Rationale
DIM 4, Structure and skills Question	Answer choice “Patient has sufficient after-hours structure to prevent risky substance use/SUD-related behaviors, but lacks sufficient skills to develop and maintain a daily structure that would enable them to prevent return to use or these behaviors during the day” has been changed to “Patient will have sufficient after-hours structure to prevent risky substance use/SUD-related behaviors (either at home or in a recovery residence), but lacks sufficient skills to create a daily structure that would keep them safe during the day”	To clarify that this answer choice allows for future environmental stability (i.e., the patient will be admitted to a recovery residence and so they will have sufficient structure at the time of admission).

Revisions to medical codes

- None

Review and Incorporation of Recent Medical Literature

Optum, Inc. is committed to keeping the InterQual product suite current and accurate. Criteria are continually reviewed and updated, with new editions of every product released at least annually. Optum's staff of physicians, nurses, other licensed healthcare professionals, and its extensive array of primary care and specialty consultants participate in ongoing criteria revision as new medical information emerges. Each release of the criteria reflects a thorough review of new medical literature, society guidelines, current practice standards, and incorporation of expert clinical consultant and user feedback.

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CDC HRQOL-14: Health-related quality of life survey, also known as the CDC HRQOL-14 or "Healthy Days Measures", is available in the public domain from the Centers for Disease Control and Prevention.

Edinburgh Postnatal depression scale: Cox, J.L., Holden, J.M., and Sagovsky, R. 1987. *Detection of postnatal depression: Development of the 10-item Edinburgh Postnatal Depression Scale*. British Journal of Psychiatry 150:782-786 .K. L. Wisner, B. L. Parry, C. M. Piontek, Postpartum Depression N Engl J Med vol. 347, No 3, July 18, 2002, 194-199

Katz Index of Independence in Activities of Daily Living Scale: Katz, S., Downs, T. D., Cash, H. R., & Grotz, R. C. (1970). "Progress in Development of the Index of ADL." *The Gerontologist*, Vol. 10 (1 Part 1), 20-30. Reproduced with permission.

Lawton Brody Instrumental Activities of Daily Living Scale: Lawton, M. P. & Brody, E. M. (1969). "Assessment of Older People: Self-maintaining and Instrumental Activities of Daily Living." *The Gerontologist*, Vol. 9 (3 Part 1), 179-186. Reproduced with permission.

MAHC-10 Tool: The Multi-Factorial Fall Risk Assessment Tool (MAHC-10) developed by the Missouri Alliance for Home Care (MAHC). Reproduced with permission.

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Six-Item Screener: Callahan CM, Unverzagt FW, Hui SL, Perkins AJ, Hendrie HC. (2002). Six-item screener to identify cognitive impairment among potential subjects for clinical research. *Medical Care*, 40(9), 771-781. Reproduced with permission.

Two Item Conjoint Screen (TICS): Brown, R. L., Leonard, T., Saunders, L. A. & Papasouliotis, O. (2001). A two-item conjoint screen for alcohol and other drug problems. *J Am Board Fam Pract*, 14, 95-106. Reproduced with permission.

Generalized Anxiety Disorder Screen (GAD-7): "A brief measure for assessing generalized anxiety disorder: the GAD-7". Spitzer RL, Kroenke K, Williams JB, Löwe B. *Arch Intern Med*. 2006 May 22; 166(10):1092-7. Doi:10.1001/archinte.166/10/1092. PMID: 16717171. Reproduced with permission.

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Merck Manual - MD Calc	https://www.merckmanuals.com/medical-calculators/CommunityAcqPneumonia-ja.htm
Social Security Actuarial Life Table (I)	https://www.ssa.gov/oact/STATS/table4c6.html
Fracture Risk Assessment Tool (FRAX)	https://www.sheffield.ac.uk/TBS/
Breast Cancer Risk Assessment Tool	https://bcrisktool.cancer.gov/
BRCAPRO	https://projects.iq.harvard.edu/bayesmendel/brcapro
CanRisk (BOADICEA)	https://canrisk.org/about/

Third Party	Link to Calculator
UNC	https://www.med.unc.edu/mfm/nips-calc
Marfan Foundation	http://www.marfan.org/dx/zscore
Marfan Foundation	https://marfan.org/dx/score/